

GENITAL SYMPTOMS

Assess the patient with genital symptoms and his/her partner/s

Assess	Note
Symptoms	Ask about genital discharge, rash, itch, lumps, ulcers and lower abdominal pain and manage as below. If anal symptoms (painless bleeding, passing mucus or difficulty passing stool) → 48.
Sexual health	If risky sexual behaviour: new or multiple partner/s, uses condoms unreliably, has sex under influence of alcohol/drugs, give safe sex advice. Ask if patient has anal sex: if anal symptoms → 48.
Abuse	Ask about sexual assault. If yes → 88.
Family planning	Assess patient's contraceptive needs → 154 and discuss infertility. Exclude pregnancy → 157.
Examination	- Woman: examine abdomen for masses, look for discharge, ulcers, rash, lumps. Do pelvic examination to check for pain on moving cervix/pelvic masses and speculum examination for cervical abnormalities. - Man: look for genital discharge, ulcers, rash, lumps, pubic lice or scrotal swelling, tenderness or masses.
HIV	Test for HIV → 110. If HIV positive, give routine care → 111. If negative, consider need for PrEP → 106.
Syphilis	- Check syphilis serology if: sexually assaulted, secondary/tertiary syphilis¹ suspected or atypical/fleshy/wet genital warts. If pregnant, test for syphilis at every visit → 162. If syphilis positive → 53. - Repeat RPR at 6 months in all treated with doxycycline/amoxicillin/probenecid.
Cervical screen	Do a cervical screen if needed → 55. If abnormal vaginal discharge, delay routine cervical screen until treated → 51. If discharge persists after treatment, do cervical screen. If cervix looks abnormal/suspicious of cancer, refer same week.

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Advise the patient with genital symptoms and his/her partner/s

- Discuss safe sex. Provide male and female condoms, advise patient to stay with one partner at a time. Offer referral for medical male circumcision.
- If patient has a sexually transmitted infection (STI), educate about cause and increased risk of HIV transmission. Urge to adhere to treatment and abstain from sex for at least 1 week after treatment.
- Stress importance of partner treatment in cure of STI: give partner notification slip with the patient's diagnosis code for each partner. Consider other notification methods like active tracing and treatment.

Treat the patient with genital symptoms

Discharge	Scrotal pain/swelling	Itch	Ulcers/sores	Lump/s	Warts
Woman → 51	Man → 50	→ 50	Discharge in woman → 51	Glans penis → 50	Pubic area → 54
			→ 52	Groin → 25	Skin → 54

Treat the partner/s according to code given on notification slip

Notification code	Treat the asymptomatic partner/s below. If partner has other STI symptoms and signs, manage as per relevant STI algorithm found on pages listed above.
VDS or LAP	Give partner single dose ceftriaxone 250mg IM ³ and azithromycin 1g orally and metronidazole ² 2g. If severe penicillin allergy ⁴ , omit ceftriaxone and increase azithromycin to 2g.
MUS or SSW	Give partner single dose ceftriaxone 250mg IM ³ and azithromycin 1g orally. If severe penicillin allergy ⁴ , omit ceftriaxone and increase azithromycin to 2g.
GUS (no discharge)	Give partner doxycycline 100mg 12 hourly for 14 days. If partner pregnant, give instead single dose benzathine benzylpenicillin 2.4MU IM ⁵ .
GUS with VDS	Give partner single dose ceftriaxone 250mg IM ³ and azithromycin 1g orally and metronidazole ² 2g. If severe penicillin allergy ⁴ , omit ceftriaxone and increase azithromycin to 2g.
GUS with MUS	Give partner single dose ceftriaxone 250mg IM ³ and azithromycin 1g orally. If severe penicillin allergy ⁴ , omit ceftriaxone and increase azithromycin to 2g.
RPR+	Test partner for syphilis: if positive → 53. If negative, give partner doxycycline 100mg 12 hourly for 14 days. If partner pregnant, give instead single dose benzathine benzylpenicillin 2.4MU IM ⁵ .
Bubo	Give partner single dose azithromycin 1g.

VDS: vaginal discharge syndrome

LAP: lower abdominal pain

MUS: male urethritis syndrome

SSW: scrotal swelling

GUS: genital ulcer syndrome

RPR+: syphilis positive result

BAL: balanitis

¹Secondary syphilis: 6-8 weeks after ulcer; generalised rash (includes palms/soles), flu-like symptoms, flat wart-like genital lesions, mouth ulcers, patchy hair loss. Tertiary syphilis: many years later; affects skin, bone, heart, nervous system. ²Advise no alcohol until 24 hours after last dose of metronidazole. ³Dissolve **ceftriaxone** 250mg in 0.9mL **lidocaine 1%** without epinephrine (adrenaline). ⁴History of anaphylaxis, urticaria or angioedema. ⁵Dissolve **benzathine benzylpenicillin** 2.4MU in 6mL **lidocaine 1%** without epinephrine (adrenaline). If benzathine benzylpenicillin unavailable, give instead **amoxicillin** 1g 8 hourly and **probenecid** 250mg 8 hourly for 14 days. If severe penicillin allergy, discuss/refer.